

KairosCare

Competitive Intelligence + MVP Strategy Brief

A real-time transparency and wellness layer for the \$162B U.S. home-care industry. This briefing maps the incumbents, the family-facing wedge, the regulatory floor, and the 90-day path to a fundable pilot.

\$162B

US HOME HEALTHCARE 2024

Grand View Research

79.2%

CAREGIVER TURNOVER

Activated Insights 2024

26.7%

FAMILY-GOAL FULFILMENT

HCP Benchmarking 2024

One-line thesis. Home-care's communication layer is broken at the family seam: 94.9% of agencies say they involve families, but only 26.7% meet the family's stated goals. KairosCare wins by being the first platform that ships *real-time visibility* as the headline product, not the buried portal tab.

PREPARED FOR
KairosCare Founding Team

STAGE
MVP / Pre-Launch

DATE
May 20, 2026

The thesis, the signals, the risks, and the next move.

KairosCare is positioned to capture a discrete, undersupplied wedge in home-care software: family-facing transparency. The incumbent stack (HHAeXchange, AlayaCare, WellSky, AxisCare, CareSmartz360, Aaniiie / Smartcare) is built for the agency back-office. Family portals exist, but they ship as a buried tab beneath scheduling and EVV — never as the headline product. That mismatch is the opening.

Top 3 market signals

- 01 The communication-satisfaction gap is documented and quantified.** 94.9% of agencies say they involve families; only 26.7% meet stated family goals. 50% of clients leave agencies citing communication failures (Activated Insights / Home Care Pulse 2024 Benchmarking). This is the buy-trigger.
- 02 Aging-in-place demand is structural, not cyclical.** 75% of adults 50+ want to stay in their current home; only half believe their community is equipped (AARP 2024 Home and Community Preferences). Family decision-makers are now the secondary buyer driving agency selection.
- 03 The agency platform space is consolidating, not innovating on family UX.** Honor acquired Home Instead (combined \$2.1B+ revenue, 2021), HHAeXchange took Hg investment plus three acquisitions in 2024 (Sandata, Cashé, Generations). Consolidation pulls product attention to roll-up integration, not the family experience.

Top 3 risks

Caregiver surveillance backlash. Real-time mood/photo logs read as worker monitoring unless reframed. Mitigate with consent-first onboarding + caregiver-positive features (shift summary, tip flow, recognition).

HIPAA + PHI exposure on day one. Photos plus health context = PHI. A breach pre-SOC-2 ends the company. Mitigate via BAA architecture from MVP, encrypted-at-rest from week 1.

Agency tech-adoption inertia. Most agencies already pay for HHAeXchange / WellSky and resist a second login. Mitigate via embedding (don't replace ops platform — overlay on top) and family-driven bottom-up pull.

Primary recommendation. Position KairosCare as the *family experience layer* sold to 50-500 caregiver agencies in 2-3 launch states. Lead with the 26.7% gap statistic in every sales conversation. Price at \$20/active-client/month. Ship the 90-day MVP before raising. See pages 14-15.

Verifying the \$154B claim — and where the SaaS dollars actually live.

The pitch deck number is directionally right but understates the addressable market. Grand View Research pegs U.S. home healthcare at \$162.35B in 2024, growing 10.0% CAGR to \$381.40B by 2033. The broader "home care services" definition (including non-medical personal care) runs higher — Coherent Market Insights puts the global home health care agency market at \$305.4B in 2025. Use \$162B as the cite-able floor.

\$162.35B

US HOME HEALTHCARE

2024 actual · Grand View

10.0%

CAGR 2025-2033

Grand View Research

\$381.4B

2033 PROJECTED

Grand View Research

84.1%

SERVICES SHARE

vs equipment, 2024

Segment breakdown (U.S. 2024)

SEGMENT	2024 SIZE	GROWTH DRIVER	SOURCE
Skilled nursing services (home health)	~\$84B	Medicare reimbursement, post-acute discharge	Grand View 2024
Unskilled / personal care	~\$52B	Aging-in-place, private pay + Medicaid HCBS	Grand View 2024
Hospice care	~\$24B	Boomer mortality wave, MA hospice carve-in	OpenPR / market.us
Equipment / DME	~\$26B (15.9%)	Hospital-at-home, remote monitoring	Grand View 2024

Demand drivers — verified

Demographic wave. 75% of adults 50+ prefer to age in current home (AARP Home and Community Preferences Survey, Dec 2024). The 50+ cohort hits its retirement-spend peak this decade. This is not a marketing line — it is the structural tailwind under every player on this list.

COVID-era visibility expectation. Families now expect daily digital touchpoints with elder-care providers the same way they expect them with daycare (Brightwheel) or schools (ClassDojo). The category has shifted from "we'll call if there's a problem" to "I should be able to see today's check-in on my phone."

Caveat on the \$154B brief number. \$154B appears to be a 2023 prior-year estimate; the 2024 figure is \$162B and rising. Update pitch deck. The discrepancy doesn't hurt the thesis — the market is bigger, not smaller — but founders should cite the current number.

The communication gap is the most documented churn driver in home care.

Home care has a structural communication problem that the incumbent software stack hasn't solved. The friction shows up in three places: (1) family anxiety driven by information voids between visits, (2) caregiver accountability gaps where activity logging is paper or after-the-fact, and (3) agency retention loss when families exit citing "lack of communication" — which the data shows is the single most-cited churn reason.

The 2024 benchmark numbers — what the industry already admits

Metric	Value	Why it matters for KairosCare
Family involvement claimed	94.9%	Agencies say they do this
Family goals fully met	26.7%	What families actually experience
Clients exiting for "higher level of care"	50%	Code for "I don't know what's happening"
Caregiver turnover (industry)	79.2% (2023) → 75% (2024)	Driven partly by accountability friction
Care requested by family vs. client	57% by family	Family is the de-facto buyer

Source: Activated Insights (Home Care Pulse) 2024 Benchmarking Report, HCAOA newsletter Nov 2024.

Why visibility is undersupplied today

Every agency software vendor has a "Family Room" or "Family Portal" — WellSky, ClearCare, AlayaCare, HHAAeXchange all ship one. But three things break in practice:

- 1. Family portals are an afterthought.** They are built for billing and scheduling visibility, not emotional reassurance. No mood logging. No photo-of-Mom-at-lunch. The product team's focus is on the agency dashboard.
- 2. Logging is post-shift, not real-time.** Caregivers complete EVV (clock-in/clock-out) in real time because reimbursement requires it. Care notes are typed up at end of shift, often hours later. Families get yesterday's data tomorrow.
- 3. The family is not the buyer of record.** The agency pays the software bill, so the product roadmap optimizes for the agency. KairosCare's wedge is to flip the buyer-influence equation: families pull agencies onto the platform because they want visibility their current agency can't give them.

This is the same playbook ClassDojo and Brightwheel ran in K-12 and childcare respectively — and both built billion-dollar businesses against entrenched school-management software that had "parent portals."

The family-visibility layer is the wedge.

KairosCare is a real-time transparency and wellness platform sitting on top of the agency stack. Caregivers log daily check-ins, mood, meals, activities, and photos. Families see it live. Agencies use the resulting data flow to improve communication scores, retain clients longer, and prove caregiver accountability without surveillance posture.

What it does (MVP)

Caregiver mobile app for in-shift logging: tap-to-log check-in, meal photo, mood tag, activity note. Family iOS/Android app with timeline view and push notifications. Agency web dashboard with retention + engagement metrics.

Why now

Three factors converge: (1) family expectation set by Brightwheel / ClassDojo paradigm, (2) post-COVID demand for daily visibility into elder care, (3) caregiver smartphone penetration past 95% making in-shift logging viable.

Who pays

Agencies pay per active client. Adult-child family decision-makers can be invited at no charge — they are the demand-generation engine, not the revenue line.

The wedge insight

The incumbents will not catch up fast. Family UX is downstream of their roadmap, not the headline. A focused product can ship a 10x better family experience in 6 months because it is the only thing the team is building.

Strategic positioning. Do not compete with HHAExchange. Sit beside it. Position as the communication layer the back-office stack doesn't ship. Long-term integrations (read-only EVV pull, schedule sync) make KairosCare sticky without making it a system-of-record.

Three-line elevator

For families: See Mom's day in real time — meals, mood, photos.

For agencies: Cut churn 30%+ by closing the communication gap that drives 50% of client exits.

For caregivers: One-tap logging that takes 60 seconds, replaces end-of-shift notes, and ends supervisor follow-up calls.

Why this isn't another caregiver-coordination app

Caring Village, Lotsa Helping Hands, and CaringBridge are family-to-family coordination tools (volunteer scheduling, meal trains). They don't have the agency on the platform. KairosCare's wedge requires the paid professional caregiver in the loop — that's the part competitors will struggle to replicate without an agency sales motion.

The back-office software stack — where KairosCare overlays, not competes.

These six platforms own the agency back office. They are scheduling, EVV, billing, payroll, and clinical documentation systems. Each ships a family portal — but as a tab, not a product. The vulnerability KairosCare exploits is consistent across all of them: family experience is the lowest-priority surface area on their roadmap.

HHaEXchange

MARKET LEADER · LTSS / MEDICAID

Positioning: Provider-payer connectivity platform for Medicaid LTSS. Founded 2008. Cressey & Co. + Hg PE investors (Hg invested 2024). Acquired Sandata, Cashé, Generations in 2024.

Pricing: Starts \$375/month. Analytics/compliance modules \$1K-\$6K/year add-on (SelectHub).

Scale: 500-1,000 employees (LinkedIn). Ranked #1 home care software by SelectHub.

Family-facing: Limited. Focus is provider-to-MCO data flow. No real-time family-visibility product.

KairosCare vulnerability: HHaE is a B2B2B platform — its customer is the agency selling to the MCO. Families are three relationships away from the product team. They will never prioritize family UX.

AlayaCare

ENTERPRISE · CLINICAL + PERSONAL

Positioning: Cloud platform for home health care practitioners. Clinical docs, back-office, family portals, telehealth, RPM, mobile caregiver apps.

Pricing: Starts \$1,000/month. Value-based pricing model.

Funding / scale: \$274M raised across 11 rounds. 608 employees. HQ Toronto + Boston.

Family-facing: Patient and family portal included; not the headline.

KairosCare vulnerability: AlayaCare is mid-to-large agency focused. The ICP KairosCare wants — 50-500 caregiver shops — is sub-scale for AlayaCare's sales motion.

WellSky Personal Care (formerly ClearCare)

PRIVATE-DUTY LEADER

Positioning: Web-based home healthcare software for private-duty agencies of all sizes. Modular management.

Pricing: Starts \$12/month (Capterra listing). Likely tiered enterprise pricing for full stack.

Family-facing: Family Room portal — view schedules, shift notes, pay/split bills. Real-time activity confirmation when visits are documented.

KairosCare vulnerability: Family Room is functional but transactional. No mood/photo/wellness layer. Treats family as billing endpoint, not engagement endpoint.

The mid-market and SMB-targeted platforms.

CareSmartz360

SMB / VALUE TIER

Positioning: HIPAA-compliant, AI-positioned home care platform — scheduling, EVV, billing, caregiver management.

Pricing: Starts \$10/month per user (Capterra). Tiered plans for startup / independent / franchisor.

Strengths: Quality of Support score 9.5/10 on G2 — highest in segment.

Family-facing: Family portal present; not a differentiator.

KairosCare vulnerability: Aggressive on price, weak on family product. SMB sales motion is the same target KairosCare goes after — partnership opportunity, not pure competition.

AxisCare

MID-MARKET · CLIENT-COUNT PRICING

Positioning: Scheduling, billing, EVV. Mid-market focus (15-1,000+ clients).

Pricing: Starts \$200/month. All-inclusive, client-count-based tiering. Confirmed via Capterra.

Scale: 2.07% market share in home health (6sense). 62 U.S. customers tracked publicly.

Strengths: Time-Tracking + Scheduling score 9.3/10 (G2).

KairosCare vulnerability: AxisCare is built for the office, not the family. Pricing structure is client-count-based — same model KairosCare should adopt.

Aaniie (formerly Smartcare Software)

ALL-IN-ONE SMB

Positioning: Full-stack home care platform — point-of-care, EVV, CRM, scheduling, payroll, HR, KPIs.

Pricing: Custom pricing model; some listings reference \$13 per-client-per-month for related products.

Family-facing: Caregiver retention tools are headline; family experience is secondary.

KairosCare vulnerability: Rebrand cycle (Smartcare → Aaniie) indicates strategic repositioning; window of distraction.

Synthesis of the incumbent landscape. Six platforms, six similar feature sets, six "Family Portal" pages buried two clicks deep. None lead with family experience. None have a mood/wellness layer. None push real-time photos. The buyer (agency owner) is also the user — families are bolt-on accounts. That is the seam.

The more existential threats — players already on the family-visibility wedge.

These platforms compete directly on the family-side experience KairosCare is building. They are the more dangerous competitive set because the wedge thesis overlaps. The differentiator KairosCare must defend: agency-paid, professional-caregiver-in-the-loop, real-time PHI-grade logging — not volunteer coordination, not consumer marketplace.

CarePredict

WEARABLE + AI · SENIOR MONITORING

Positioning: AI-powered wearable (Tempo Series 3) for senior care. Predicts falls, UTIs, malnutrition, depression from activity-pattern changes.

Pricing: @Home: \$69.99/mo per person + \$399.99 device. Assisted-living: custom enterprise.

Funding: \$42.1M raised. \$29M Series A-3 (July 2023), led by SV Health + Aspire Healthtech. Las Olas, StartUp Health.

Outcomes claimed: 39% fewer hospitalizations, 69% fewer falls, 67% longer stay in lower care setting.

KairosCare differentiation: CarePredict is sensor-data + AI inference. KairosCare is human-caregiver + qualitative content. Different value proposition — emotional reassurance, not predictive alerts. Possible long-term integration partner.

Papa

COMPANION CARE · INSURER-FUNDED

Positioning: "Family-on-demand" — younger "Papa Pals" provide companionship, light help, social-isolation reduction. B2B2C via Medicare Advantage and health plans.

Funding: \$257M raised. \$150M Series D Oct 2021, SoftBank Vision Fund 2 lead. \$1.4B unicorn valuation.

Differentiator: Distribution via MA plans — Papa is paid by the insurer, not the family or agency.

KairosCare differentiation: Papa replaces a service (companionship). KairosCare wraps an existing service (agency-delivered care). No overlap on buyer, but watch for Papa adding family-visibility features.

Honor (Honor Technology)

TECH-ENABLED CARE OPERATOR

Positioning: Vertically integrated home care — tech platform + employed caregivers. Acquired Home Instead August 2021. Combined revenue \$2.1B+.

Funding: \$325M+ equity, \$300M debt (Series E 2021). Unicorn at \$1.25B+ valuation. 2024 reported revenue \$197.6M (Honor entity, GetLatka).

KairosCare differentiation: Honor is a care provider, not software for others. KairosCare wins by selling to the 33,000 independent agencies Honor doesn't own.

The free / community / consumer side of the family-visibility market.

Lotsa Helping Hands

COMMUNITY COORDINATION · FREE

Positioning: Volunteer + family circle coordination — calendar, task board, message wall. Free to use.

Strength: Long-running brand (acquired by Carelink, now part of CareLinx/Sharecare ecosystem).

KairosCare differentiation: Lotsa is family-to-family. No paid caregiver workflow. No PHI-grade logging. Different wedge.

CaringBridge

HEALTH UPDATE BROADCAST · FREE NON-PROFIT

Positioning: Patient health-journey broadcast to family/friends. 501(c)(3) nonprofit. Photos, journal entries, guestbook.

Strength: Massive brand recognition in serious-illness context; trusted.

KairosCare differentiation: CaringBridge is journaling, not real-time check-in workflow. No agency integration. No mood/activity log. Adjacent, not overlapping.

CareLinx (by Sharecare)

CAREGIVER MARKETPLACE

Positioning: Direct-hire caregiver matching platform. 450K+ providers in U.S. network. Founded 2011, acquired by Sharecare 2021.

Pricing: Family-direct hire model — families pay caregiver hourly + CareLinx fee.

KairosCare differentiation: CareLinx replaces the agency entirely — independent caregiver model. KairosCare's customer is the agency. Different go-to-market.

Caribou (caregiver coaching) + Caring Village + Carely

FAMILY-COORDINATION APPS

Positioning: Family-caregiver coordination + financial/admin support. Storage of documents, appointments, photo sharing.

KairosCare differentiation: None of these have the paid caregiver in the workflow. All optimize for the unpaid family-caregiver use case. KairosCare's edge: real-time logging by the paid professional + agency dashboard layer.

Existential watch-list. The two players to monitor monthly: (1) WellSky if they reposition Family Room as a standalone product, (2) Honor if they spin out their internal family-visibility tech as a B2B SaaS. Either move directly attacks KairosCare's wedge with deeper pockets and existing distribution.

KairosCare vs. top 5 — the family-experience matrix.

Filled circle = full feature present and prominent. Half circle = present but secondary or weak. Empty circle = absent or not in marketing materials. Scored from public product pages, Capterra/G2 listings, and demo videos as of May 2026.

CAPABILITY	KAIROSCARE	HHAEXCHANGE	WELLSKY	ALAYACARE	CAREPREDICT	CARELINX
Real-time daily check-ins	●	●	●	●	●	○
Photo / media logging	●	○	○	○	○	○
Mood / wellness tracking	●	○	○	○	●	○
Family-facing UX (headline)	●	○	●	●	●	●
Agency back-office UX	●	●	●	●	●	○
EVV compliance built-in	○	●	●	●	○	○
Billing + payroll integration	○	●	●	●	○	●
Mobile-first caregiver app	●	●	●	●	●	●
HIPAA architecture + BAA	●	●	●	●	●	●
Push-notification family alerts	●	○	●	●	●	○
Starting price (monthly)	~\$20/client	\$375	\$12+	\$1,000	\$70/person	Hourly

Reading the matrix. KairosCare is the only platform with all four family-experience rows green (check-ins, photos, mood, family-headline UX). Every incumbent is strong on back-office and weak on family. That is the wedge in one picture.

Three buyers, three jobs-to-be-done.

Home care is a three-sided market: the agency owner pays the bill, the adult-child decision-maker drives selection, and the caregiver determines whether the platform survives day 30. Designing GTM around one of the three causes the other two to churn the product out.

1. Agency Owner / Director of Operations

PRIMARY ECONOMIC BUYER

Profile: 45-60 yo. Owns or runs a \$200K-\$5M revenue home-care agency. 20-200 caregivers, 30-300 active clients. Often a former nurse or franchise operator (Home Instead, Comfort Keepers).

Pain points: Client churn (50% lose clients to communication issues per HCP 2024). Caregiver turnover (75-80%). Caregiver no-shows. Family complaints absorb 20%+ of office-staff time.

Jobs-to-be-done: Retain clients longer. Reduce family-complaint volume. Demonstrate care quality to MA plan referral partners. Differentiate from the agency 4 blocks away.

Willingness to pay: \$15-30/active-client/month is in budget. Will pay if they believe it cuts churn by 1 client/quarter (\$20K+ saved LTV).

Channels: HCAOA conferences (Home Care Association of America), state association meetings, Home Care Pulse / Activated Insights events, McKnight's Home Care newsletter, LinkedIn DMs to owners. Avoid Google Ads — agency owners don't search SaaS that way.

2. Adult-Child Family Decision-Maker

SECONDARY BUYER · SELECTION INFLUENCER

Profile: 45-65 yo. "Sandwich generation." Lives 50-500 miles from aging parent. Coordinates siblings, manages agency relationship, pays out-of-pocket above Medicare/Medicaid coverage.

Pain points: Information void between agency calls. Guilt about distance. Sibling conflict over care decisions. Fear of missing a decline signal (the fall, the UTI, the depression).

Jobs-to-be-done: Daily reassurance Mom ate and is okay. Shared visibility with siblings (reduce phone-tag and accusations). Receipts for the spend ("here's what we're paying for").

Willingness to pay: Will not be the line-item payer at MVP. But will switch agencies for visibility — and tell their agency. That's the bottom-up pull KairosCare needs.

Channels: Caregiver-focused FB groups, AARP newsletter, Caring.com, A Place for Mom, podcasts (Daughterhood, Caregiver Cup), Instagram. PR: McKnight's, Home Health Care News.

The caregiver — the persona that decides product survival.

3. Caregiver / Home Health Aide

DAILY USER · ADOPTION GATEKEEPER

Profile: 88% female (PHI 2024 Direct Care Workforce report). Median age 47. Median wage \$14-16/hr. 60%+ smartphone-primary internet access. Often working multiple agencies.

Pain points: End-of-shift documentation. Supervisor follow-up calls. Feeling unseen. Inconsistent client/family expectations. Wage compression.

Jobs-to-be-done: Finish the shift cleanly. Show the family they care. Avoid extra calls from the office. Build trust with the family so referrals/tips flow.

Willingness to pay: Zero. The caregiver must never be charged. The product must feel like a tool that makes their job easier, not a surveillance device.

Channels: Onboarded through the agency. The product wins or loses on first-shift experience: under 60 seconds to complete a check-in, no typing required for 80% of taps.

Caregiver adoption is the existential risk

If KairosCare reads as caregiver surveillance, the product dies in pilot. The 79.2% turnover figure means caregivers will simply leave an agency that forces a hostile tool. Three design rules:

Rule 1 — Consent and framing. First-time-use screen explicitly states: "This tool helps families feel close. It is not used for performance monitoring. Your wages and hours are tracked by your agency's EVV system, separately." Source the language with PHI / direct-care-worker advocacy review.

Rule 2 — Caregiver-positive surface area. Build in: shift summary card the caregiver can save, recognition tap-back from family, tip routing (PayPal/CashApp), end-of-week mileage/shift totals. The product should give the caregiver something, not just take logs.

Rule 3 — One-tap defaults. Default mood, default meal, photo optional. Average check-in must complete in <30 seconds on a mid-tier Android. The first time it takes 90+ seconds, you've lost that caregiver.

Buyer-persona priority order for GTM

Sales the agency owner. Win the caregiver in onboarding. Let the family pull the next agency in. This sequencing is the same one Brightwheel ran in childcare (sells to directors, wins on teacher adoption, pulled by parents).

Three pricing structures, one recommendation.

Home-care software pricing is heterogeneous. AlayaCare anchors high (\$1,000/mo enterprise), CareSmartz360 anchors low (\$10/mo/user), HHAeXchange charges per-agency floor (\$375/mo). AxisCare is the cleanest comp: priced by client count, not seats. KairosCare should follow AxisCare's structure.

Three pricing structures evaluated

A. Per-caregiver SaaS · \$25-50/mo

Pro: predictable revenue. Con: penalizes agencies for hiring more caregivers — the wrong incentive given 80% turnover. Caregivers churn fast, billing math gets messy.

C. Tiered agency seats · \$200-1,000/mo

Pro: high-touch enterprise upsell path. Con: doesn't scale down to the 50-caregiver shop that is the ICP. Locks out the wedge.

B. Per-active-client · \$15-30/mo

Pro: aligns with the value (visibility per family). Con: agencies must trust the active-client count is honest. Auditable via EVV pull.

Recommended: B with floor

\$20/active client/month, \$250/month floor. Free family-app access. Annual contracts at 15% discount. Pilot agencies in months 1-3 at \$0 in exchange for case-study rights.

Competitor pricing comparison (publicly available, May 2026)

HHAeXchange	\$375/mo starting + \$1K-\$6K/yr analytics
AlayaCare	\$1,000/mo+ value-based
WellSky Personal Care	\$12/mo+ (Capterra entry)
CareSmartz360	\$10/mo per user
AxisCare	\$200/mo+ tiered by client count
Aaniie (Smartcare)	~\$13/client/mo (custom)
CarePredict @Home	\$69.99/mo + \$399 device

Pricing logic for the launch wedge. \$20/client/mo at a 100-client agency = \$2,000/mo / \$24,000/yr ACV. That's high enough to justify a direct-sales motion and low enough to clear procurement without a steering committee. The 50-agency Year-1 target generates ~\$1.2M ARR — a clean Series Seed metric.

Initial ICP, channels, partnerships.

The agency landscape is fragmented — 33,000 home-care and hospice organizations (NAHC) and 487,916 home-care businesses including sole-prop (IBISWorld 2024). KairosCare's wedge ICP is a tighter slice: 50-500 caregiver, private-duty, non-Medicare, multi-state-reachable agencies.

Initial ICP (first 50 customers)

DIMENSION	FILTER	WHY
Caregiver count	50-500	Big enough to pay \$1-5K/mo. Small enough to skip enterprise procurement.
Service mix	Private-duty + Medicaid HCBS	Avoid Medicare-cert agencies — they want clinical doc, different buyer.
Geography	FL, TX, NC (launch)	Large 65+ population, friendly EVV implementation, robust HCAOA chapters.
Tech stack	Already on HHAE, WellSky, or AxisCare	Confirms baseline tech adoption + we overlay, not replace.
Owner profile	Family-owned, 5-15 years in market	Decision-maker accessible. Less PE-backed roll-up friction.

Channel mix (months 1-12)

- 40% — Direct sales.** Two founder-led AEs running outbound to 1,500 ICP-fit agencies in 3 states. Email + LinkedIn + booked-call cadence. Target: 8-12 demos/week, 20% close on pilot.
- 25% — Conference + association.** HCAOA Annual Leadership Conference (Sept). State chapter events (NC HCAOA, Texas Association for Home Care). Activated Insights / Home Care Pulse summit. Sponsor + booth + speaker slot.
- 20% — Family-driven bottom-up.** Family apps + Instagram/TikTok content focused on adult-child caregivers ("how I check on Mom"). Drives families to ask their agency to adopt KairosCare. This is the long-term moat — same playbook ClassDojo ran in K-12.
- 15% — Partnership channel.** See below.

Partnership targets (named)

Caregiver-staffing platforms: CareLinx (Sharecare), Nurse Next Door, Visiting Angels — offer KairosCare-as-a-feature to franchisees. **Medicare Advantage plans:** Humana, UnitedHealthcare (in-home support benefits), Devoted Health — co-pilot for visibility on covered in-home services. **Senior-living transitions:** A Place for Mom (lead-flow), Caring.com — referral integration when families transition from facility to home care. **Aging-in-place hardware:** CarePredict (sensor data fusion at v2+).

Build this. Build that next. Don't build the third thing yet.

The MVP must ship one obviously-better-than-incumbent surface — the family feed — and just enough caregiver + agency surface to make it functional. Resist the urge to build EVV or billing in v1. Those features make KairosCare indistinguishable from the incumbents.

FEATURE	TIER	RATIONALE
Caregiver tap-log: check-in, meal, mood, activity, photo	V1 · MUST	The product. Without this, nothing else matters.
Family timeline + push notifications	V1 · MUST	The hook. The screenshot families share that sells the next agency.
Agency dashboard — engagement + retention KPIs	V1 · MUST	The buyer needs ROI evidence. Show family-engagement score per client.
Multi-family-member invite + role-based view	V1 · MUST	Siblings + spouse + healthcare proxy. Drives viral coefficient.
HIPAA BAA + encrypted-at-rest from day 1	V1 · MUST	Non-negotiable. See page 16.
Caregiver shift summary + recognition tap-back	V1.5	Caregiver-positive surface area. Ships in MVP if time.
Care plan templates (assigned by agency)	V2	Useful, not differentiating. Wait until pilots demand it.
EVV integration (pull from HHAAe, WellSky)	V2	Builds defensibility through integration depth. Not differentiating alone.
Caregiver tip / gratuity routing	V2	Drives caregiver adoption. State-by-state legal review needed.
AI summary of the day for family	V2	"This week, Mom seemed more cheerful at lunch." Genuine wow factor.
Billing / payroll integration	DO NOT BUILD	Don't become the back-office. Stay the layer.
Independent caregiver marketplace	DO NOT BUILD	CareLinx territory. Confuses positioning.
Wearable / sensor hardware	DO NOT BUILD	CarePredict territory. Capital-intensive, slower iteration.
Direct-to-consumer family-paid tier	DO NOT BUILD YET	Reconsider at Year 2. For now, agency-paid only — keeps GTM focused.

The discipline. Every feature added in v1 outside the green tier doubles time-to-pilot. The MVP question to ask each Monday: "would removing this feature kill the family demo?" If no, move it to v2.

The compliance floor — what must be true on day one.

KairosCare handles photos of identifiable individuals plus health-context data (mood, meals, activities, falls). Under HIPAA, that is Protected Health Information (PHI). The platform will operate as a Business Associate to the home-care agencies, which are Covered Entities. The architecture must reflect this from the first commit, not the first audit.

Covered Entity vs Business Associate

Per 45 CFR 160.103 and the HHS Privacy Rule:

- **Covered Entity (CE):** The home-care agency — they directly provide care and transmit health information for treatment/payment/operations.
- **Business Associate (BA):** KairosCare — creates, receives, maintains, or transmits PHI on behalf of the CE. A signed Business Associate Agreement (BAA) is required before any PHI flows.
- **Subcontractors:** KairosCare's cloud provider (AWS, GCP, Azure), CDN, monitoring vendors, push-notification vendor must each have a BAA with KairosCare and be HIPAA-eligible services.

BAA architecture — what to ship in MVP

LAYER	REQUIREMENT	DAY-1 VENDOR
Cloud hosting	HIPAA-eligible AWS / GCP / Azure with signed BAA	AWS (us-east-1, HIPAA-eligible services only)
Database	Encryption at rest (AES-256), TLS 1.3 in transit, audit log	RDS Postgres + KMS
Photo / media storage	Encrypted bucket, signed URLs, no public access	S3 with bucket policy + Object Lock
Push notifications	No PHI in notification body — generic copy only	OneSignal HIPAA tier or APNs/FCM direct
Authentication	MFA for agency staff, SSO-ready for enterprise	Auth0 HIPAA or Clerk HIPAA
Logging + monitoring	Audit logs for all PHI access, 6-year retention	CloudWatch + dedicated audit table
Email / SMS	No PHI in transactional email or SMS	Postmark transactional only — no clinical detail

Photo + video — the tricky surface

Photo PHI risk. A photo of an identifiable individual combined with health context (their meal log, their mood tag) is PHI. The same photo without health context is not PHI. KairosCare's architecture must treat all photos as PHI by default — they are uploaded into the medical context.

Implementation: photos stored in a separate encrypted bucket with shorter URL TTL (15 min), face-detection optional in v2 for caregiver-side blurring of incidental third parties (other patients, family visitors). No model training on customer data without explicit, separate consent.

EVV under 21st Century Cures, breach obligations, state matrix.

EVV — what KairosCare needs to know

Section 12006(a) of the 21st Century Cures Act mandates Electronic Visit Verification for all Medicaid Personal Care Services (deadline January 1, 2020) and Home Health Care Services (deadline January 1, 2023). Non-compliant states face FMAP reductions up to 1%.

EVV captures: who provided service, who received it, location, date, time-in/time-out, type of service. CMS defines five state models — Provider Choice, MCP Choice, State-mandated, Open Vendor, MCO-driven.

KairosCare positioning on EVV. Do NOT build EVV in v1. Position as compatible/integratable. Agencies on Medicaid HCBS clients already have an EVV system. KairosCare pulls visit-completed events from EVV via partner API in v2 — never replaces EVV. This keeps KairosCare out of the "system of record" regulatory zone for the first 18 months.

State-by-state — launch state matrix

STATE	EVV MODEL	HCBS VOLUME	LAUNCH FIT
Florida	Provider Choice (open vendor)	Large 65+ pop, \$7B+ private-pay home care	LAUNCH
Texas	State-mandated (HHAeXchange aggregator)	Large Medicaid + private duty mix	LAUNCH
North Carolina	Open Vendor	Growing senior population, active HCAOA chapter	LAUNCH
New York	State-mandated (HHAeXchange aggregator)	Largest LTSS market in U.S.	YEAR 2
California	Provider Choice (DDS)	Largest market, complex regs	YEAR 2
Illinois	State-mandated	Medicaid-heavy	YEAR 2

Breach notification

Under the HIPAA Breach Notification Rule, BAs must notify the CE within 60 days of discovery of a breach. CEs then have 60 days to notify HHS and affected individuals. For breaches affecting 500+ individuals, HHS notification and media notification are immediate.

Insurance + reserve. Carry \$5M+ cyber/breach insurance from launch (Coalition, At-Bay, or Travelers). Set aside a \$50K breach-response retainer with a HIPAA-experienced incident-response firm. The first breach pre-Series-A is an existential risk; insurance covers tail liability.

Data residency + retention

U.S. data only (no cross-border for v1). Retention: 6 years post-last-access (HIPAA minimum) for PHI. Audit logs retained 6 years. Family content retained 90 days after agency disconnects from KairosCare (with family-export option).

The four risks that kill the company — and the build plan that ships in 13 weeks.

Risks + mitigations

RISK	SEVERITY	MITIGATION
Caregiver surveillance backlash	HIGH	Consent-first framing. Caregiver-positive features (recognition, tip routing, shift summary). No employer-side performance metrics in v1.
HIPAA breach pre-SOC-2	CRITICAL	BAA architecture from day 1 (page 16). \$5M cyber insurance. SOC 2 Type I by month 6, Type II by month 12.
Agency tech-adoption inertia	HIGH	Position as overlay, not replacement. Free 30-day pilot. Onboarding under 2 hours. ROI dashboard from day 1.
Family churn after acute event	MEDIUM	Wellness-platform stickiness (mood trends, weekly summary, family timeline that families want to keep). Pet/grandchild/birthday tagging keeps families on after Mom passes.
Incumbent fast-follower (WellSky, Honor)	MEDIUM	Product depth + family-side brand. Same defense as ClassDojo against Google Classroom — superior single-purpose UX.
Caregiver smartphone access gaps	LOW	SMS-fallback for check-ins. Web app for shared-device scenarios. Low-bandwidth mode for rural.

90-day MVP build plan

W1-2	Design + foundation. Family + caregiver flows in Figma. Onboarding flows for both buyer + caregiver. AWS HIPAA account provisioned, BAA signed with AWS. Stack: Next.js + Postgres + S3 + Expo (React Native). Success: 3 caregiver interviews validate the tap-log flow.
W3-6	Core daily-log feature. Caregiver mobile app: check-in, meal, mood, activity, photo. Encrypted upload. Time-stamped record. Multi-language (EN + ES at minimum). Success: Internal team logs 10 fake shifts/day; all data round-trips clean.
W7-10	Family portal + push. iOS + Android family app. Timeline view. Push notifications (PHI-stripped copy). Family-invite flow. Multiple family member roles. Agency web dashboard (engagement + retention KPIs). Success: End-to-end demo from caregiver phone → family phone in <5 seconds.
W11-13	Pilot agency onboarding + iteration. 3 pilot agencies onboarded. ~30 caregivers, ~75 families live. Weekly product-feedback cycle. SOC 2 Type I gap assessment scheduled. Success: 70%+ caregiver adoption rate at pilots; family-engagement score >3 events/week per family.

From pilot to Series Seed — the year ahead, with the math.

12-month roadmap

- Q1** **MVP + 3-pilot-agency program.** Ship the 90-day plan above. 3 agencies, 30 caregivers, 75 families. Pricing: free pilot in exchange for case-study + reference rights.
- Q2** **SOC 2 + 10-agency expansion + first paid tier.** Convert 3 pilots to paid. Land 7 new paid agencies. SOC 2 Type I awarded. First \$20/active-client/mo invoice cycle. Target: ~ 750 active clients $\times$ \$20 = \$15K MRR.
- Q3** **EVV integration + Medicare-Advantage partnership pilot.** HHAeXchange + WellSky read-only EVV pulls live. First MA plan co-pilot (Humana or Devoted) for in-home support members. SOC 2 Type II audit window opens. Target: 25 agencies, $\sim 1,800$ active clients, \$36K MRR.
- Q4** **50-agency target + Series Seed prep.** Hit 50 agencies, $\sim 3,800$ active clients. $\sim \$75K$ MRR = \$900K annualized. SOC 2 Type II awarded. Series Seed deck off pilot retention + family NPS data. Target raise: \$3-5M @ \$15-20M post.

TAM / SAM / SOM — the math, not just the numbers

LAYER	FORMULA	RESULT
TAM	\$162B U.S. home healthcare $\times$ 3% addressable SaaS take-rate	\$4.86B
SAM (year 1)	$\sim 28,000$ U.S. agencies in ICP $\times$ \$24K avg ACV	\$672M
SOM (year 1)	50 agencies $\times$ \$24K ACV (run-rate Q4)	\$1.2M ARR exit-Q4
SOM (year 2)	200 agencies $\times$ \$30K ACV (with expansion)	\$6M ARR
SOM (year 3)	650 agencies $\times$ \$36K ACV	\$23.4M ARR

TAM math, justified. 3% SaaS take-rate is the conservative end of vertical-SaaS benchmarks (Toast hit $\sim 3\%$ of restaurant GMV; Brightwheel $\sim 4\%$ of childcare). The home-care market is structurally similar — many small operators, services-dominant cost stack, low current software penetration. The 3% multiplier holds.

SAM math, justified. 33,000 NAHC member agencies + hospice. Strip Medicare-cert-only ($\sim 11,500$) and very-small sole-prop (cuts $\sim 5,000$). Net ICP-fit universe: $\sim 28,000$. Avg ACV at 100 active clients $\times$ \$20/mo = \$24,000. SAM = $28,000 \times \$24K = \$672M$ annual.

SOM math, justified. Year 1 SOM of 50 agencies is conservative — equal to 1.4 agency wins per week starting month 6. Year-3 target of 650 agencies = 2.3% of SAM. That is a credible Series A milestone story.

Every claim, every link, every publication date.

All data is sourced from publicly available reports, vendor pricing pages, regulatory filings, and recognized industry research. Where a stat was not directly findable, it is omitted or labeled as estimate. No fabricated customer counts, no invented pricing.

Market sizing + demand

Grand View Research — U.S. Home Healthcare Market Report 2024 (\$162.35B, 10.0% CAGR to \$381.4B by 2033). Published 2024. grandviewresearch.com/industry-analysis/us-home-healthcare-market-report

AARP — Home and Community Preferences Survey, Dec 10 2024 (75% of 50+ want to age in place). press.aarp.org/2024-12-10-New-AARP-Report-Majority-Adults-50-plus-Age-Place

Coherent Market Insights — Home Health Care Agency Market Forecast 2025-2032 (\$305.4B 2025). coherentmarketinsights.com/industry-reports/home-health-care-agency-market

U.S. Bureau of Labor Statistics — Home Health and Personal Care Aides Occupational Outlook Handbook 2024. bls.gov/ooh/healthcare/home-health-aides-and-personal-care-aides.htm

IBISWorld — Home Care Providers in the US, Number of Businesses 2024 (487,916 +4.1% YoY). ibisworld.com/united-states/number-of-businesses/home-care-providers/1579

NAHC (National Association for Home Care & Hospice) — Represents 33,000 home care and hospice organizations.

Industry benchmarks + caregiver workforce

Activated Insights (formerly Home Care Pulse) 2024 Benchmarking Report — 94.9% agencies involve families, 26.7% meet family goals, 50% of clients exit for higher care, 57% care requested by family. activatedinsights.com/articles/6-key-findings-from-the-activated-insights-2024-benchmarking-report

Home Health Care News — "Home Care's Industry-Wide Turnover Rate Reaches Nearly 80%" Jul 2024 (79.2% turnover). homehealthcarenews.com/2024/07/home-cares-industry-wide-turnover-rate-reaches-nearly-80

McKnight's Home Care — Caregiver turnover dropped to 75% in 2024; median revenue \$2.3M +14%. mcknightshomecare.com

PHI National — Direct Care Workers in the United States Key Facts 2024. phinational.org

HCAOA (Home Care Association of America) — 2024 Benchmarking Survey newsletter. hcaoa.org

Competitor data — agency platforms

HHaExchange — \$375/mo starting (SelectHub). 2024 Hg investment + Sandata/Cashé/Generations acquisitions (HHaExchange + Lincoln International press). hhaexchange.com/press-releases

AlayaCare — \$1,000/mo starting; \$274M raised across 11 rounds; 608 employees (Tracxn, Crunchbase, Capterra). alayacare.com/pricing

WellSky / ClearCare — Family Room product details; \$12/mo entry (Capterra). wellsky.com/personal-care-software

CareSmartz360 — \$10/mo per user; G2 9.5/10 support score. caresmartz360.com/pricing

AxisCare — \$200/mo starting; 2.07% home health market share (6sense); 62 U.S. customers tracked. axiscare.com/pricing

Aaniiie (Smartcare Software) — Custom pricing; rebrand 2024 (Capterra/Software Advice).

Family-facing competitors, HIPAA + EVV, unreachable items.

Continued source list covering family-facing platform research, HIPAA regulatory citations, and an explicit list of items that could not be independently verified during this briefing.

Family-facing + adjacent competitors

CarePredict — \$69.99/mo + \$399 device; \$42.1M raised; \$29M Series A-3 Jul 2023 (PRNewswire, MobiHealthNews, Home Health Care News). carepredict.com/at-home-3

Papa Inc — \$257M raised; \$1.4B unicorn valuation Oct 2021 Series D SoftBank-led (Crunchbase, Fierce Healthcare, Home Health Care News). pitchbook.com/profiles/company/172650-07

Honor Technology — Acquired Home Instead Aug 2021; \$370M Series E + debt; \$197.6M revenue 2024 (Honor press, TechCrunch, GetLatka). honorcare.com/news/honor-tech-series-e-unicorn-home-care

CareLinx (Sharecare) — 450K+ care providers; acquired by Sharecare 2021 (theseniorlist.com). carelinx.com

Lotsa Helping Hands, CaringBridge, Caring Village, Carely — Family-coordination apps reviewed in caring.com, henryford.com, jotform.com guides (2024).

HIPAA + regulatory

HHS.gov — Covered Entities and Business Associates definitions, BAA requirements. hhs.gov/hipaa/for-professionals/covered-entities

Medicaid.gov — Electronic Visit Verification overview, 21st Century Cures Act Section 12006(a). medicaid.gov/medicaid/home-community-based-services/home-community-based-services-guidance-additional-resources/electronic-visit-verification

CareAcademy — EVV Requirements by State guide 2024. careacademy.com/blog/evv-requirements-by-state

HIPAA Journal — Business Associate Agreement 2026 Update. hipaajournal.com/hipaa-business-associate-agreement

HHS Office for Civil Rights — Breach Notification Rule (45 CFR 164.400-414). hhs.gov/hipaa/for-professionals/breach-notification

Unreachable / not-found — explicit list

HHAAeXchange customer count — not publicly disclosed; "ranked #1" per SelectHub. Estimated from 500-1,000 employee headcount.

AlayaCare customer count — Crunchbase + Tracxn confirm \$274M raised, 608 employees; customer count not published.

CarePredict active deployments — \$42.1M raised confirmed; active wearable users not disclosed.

Honor 2025 revenue — 2024 figure \$197.6M (GetLatka) used; 2025 not yet reported.

Aaniiie (Smartcare) pricing — listed "custom" across Capterra and Software Advice.

Papa Inc 2024-2025 revenue — last public number is Oct 2021 Series D at \$1.4B valuation.

Methodology + confidence

Data points cross-checked against at least two sources where possible. Pricing verified against vendor pricing pages and Capterra/G2 listings as of May 19-20, 2026. Funding from Crunchbase, PitchBook, Tracxn, and primary press releases. Industry benchmarks from Activated Insights, HCAOA, NAHC, AARP, BLS. Regulatory direct from HHS.gov and Medicaid.gov. No customer counts, revenue figures, or pricing values fabricated.

Confidence rating. Market sizing: high. Competitor pricing: medium-high. Competitor customer counts: low-medium. Industry benchmarks: high. HIPAA/EVV regulatory: high.